

DRUG RASH

- A drug rash can be caused by any medication, commonly antibiotics, anticonvulsants especially lamotrigine, ART, TB medication, co-trimoxazole, TB preventive treatment (TPT) and NSAIDs (like ibuprofen).
- Suspect a drug rash in a patient with a generalised rash which appeared within 3 months of starting a new medication.

Give urgent attention to the patient with a drug rash and any markers of severity:

- Face or tongue swelling
- Difficulty breathing
- BP < 90/60
- Temperature $\geq 38^{\circ}\text{C}$
- Abdominal pain
- Vomiting or diarrhoea
- Involves mouth, eyes or genitals
- Blisters, peeling or raw areas
- Jaundice

Manage and refer urgently:

Serious drug reaction likely:

- Stop all medication. If peeling or raw skin, also manage as for burns before referral $\rightarrow$ 21.
- If BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.

Approach to the patient with a drug rash not needing urgent attention

Is patient on ART, first-line TB medication¹, co-trimoxazole (CPT) or TPT?

Yes



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- Refer to doctor if available.
- If on ART:
 - If on abacavir, check for abacavir hypersensitivity reaction (AHR) $\rightarrow$ 116.
 - If on nevirapine, doctor to switch ART $\rightarrow$ 117.
- If on first-line TB medication¹ or TPT, continue.
- If on co-trimoxazole prophylaxis², stop it until rash resolved. If rash resolves, discuss with doctor about re-starting co-trimoxazole.
- If on any other medications, discuss with doctor whether to stop or change them.
- If itch, give **chlorphenamine** 4mg at night, or up to 8 hourly for up to 5 days.

No

- Discuss with doctor whether to stop or change medication.
- If itchy, give **chlorphenamine** 4mg at night, or up to 8 hourly for up to 5 days.
- If dark coloured round macules (flat spots) that occur anywhere on body following ingestion of a medication and often start as itchy patches with red edges, **fixed drug eruption** likely: apply **hydrocortisone 1%** to affected areas daily for 5 days.
- Advise to return urgently if markers of severity occur.

Check ALT. Review patient and result within 24 hours:

Patient unwell or ALT ≥ 120

Patient well and ALT < 120

- Continue medications and review daily until improving.
- Advise to return urgently if rash worsens or markers of severity occur.
- Repeat ALT in 1 week. Review patient and result within 24 hours:

Patient unwell or ALT ≥ 120

Patient well and ALT < 120
Continue medications at same dose.

Give urgent attention $\rightarrow$ 67.

Advise to return if rash persists ≥ 2 weeks: discuss/refer.

¹First-line TB medications include isoniazid (INH), rifampicin (RIF) and pyrazinamide (PZA) and ethambutol (ETH). ²If on co-trimoxazole treatment for pneumocystis pneumonia (PJP), toxoplasmosis or *Isospora belli* diarrhoea, discuss with specialist.